4.10

PAEDIATRICS

Skin and Soft Tissue Infections

CHAPTER 4.10	SKIN AND SOFT TISSUE INFECTIONS		
Infections and Likely Organisms	**Suggested Treatment**		**Notes**
	Recommended	**Alternative**	
Cellulitis			
Furuncle, carbuncle, abscess formation *Staphylococcus aureus*	**Mild:** Cloxacillin 15 mg/kg/dose PO q6H	**Mild:** Cefalexin 25 mg/kg/dose PO q12H	• Incision and drainage (I&D) is the mainstay of therapy for moderate to severe abscess. • If allergic to penicillin: Vancomycin 15 mg/kg/dose IV q8H.
	Moderate to severe: Cloxacillin 50 mg/kg/dose IV q6H	**Moderate to severe:** Co-amoxiclav 25 mg/kg/dose (as Amoxicillin) PO q8H	
Impetigo			
Non-bullous impetigo *Streptococcus* GAS	Topical Mupirocin 2% q8H **OR** Topical Fusidic Acid 2% q8H for 7-10 days		• Consider nasal swab to detect nasal carriage in recurrent cases.

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Bullous impetigo *S aureus* (MSSA/MRSA): non-exfoliative toxin producers	Cloxacillin 15 mg/kg/ dose PO q6H	Cefalexin 25 mg/kg/dose PO q12H	• Topical Mupirocin ointment 2% q8H **OR** Topical Fusidic Acid cream 2% q8H for mild infection. • CA-MRSA: Clindamycin 7.5-10 mg/kg/dose PO q8H. • If penicillin allergy: Vancomycin 15 mg/kg/dose IV q8H.
Erysipelas *Streptococcus spp* (Groups A, B, C, G) MSSA MRSA (rare)	**Mild:** Phenoxymethylpenicillin 15-25 mg/kg/dose PO q6H	**Mild:** Amoxicillin 15-25 mg/kg/ dose q8H **OR** Cefalexin 25 mg/kg/dose q12H	• Total duration of treatment: 10 days.
	Moderate: Benzylpenicillin 25,000-50,000 units/ kg/dose IV q6H	**Moderate:** Co-amoxiclav 25 mg/kg/ dose (Amoxicillin component) IV q8H	
Scalded Skin Syndrome			
Staphylococcus aureus (MSSA)	Cloxacillin 50 mg/kg/ dose IV q6H	Cefazolin 30 mg/kg/ dose IV q8H	
Necrotising Fasciitis			

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Streptococcus spp. *Pseudomonas* spp. ***Polymicrobial:*** *Clostridium perfringens* *Bacteroides fragilis*	Meropenem 20 mg/kg/dose IV q8H + Clindamycin 10 mg/kg/dose (max 1.2 g) IV q6-8H	Piperacillin/Tazobactam 100 mg/kg/dose (Piperacillin component) IV q8H + Clindamycin 10 mg/kg/dose (max 1.2 g) IV q6-8H	• NF in children can follow post varicella zoster infection. • Can present with Streptococcal toxic shock syndrome. • Pseudomonas infection often follows burn or trauma. • Antibiotic plus urgent surgical debridement is often necessary in severe cases.
Animal Bites: Dogs and Cats			
Pasteurella multocida *S aureus* *Streptococcus* spp Anaerobes *Capnocytophaga* sp *(often multimicrobials)*	Co-amoxiclav 25 mg/kg/dose (Amoxicillin component) IV q8H **Severe:** Piperacillin/Tazobactam 100 mg/kg/dose (Piperacillin component) IV q8H ± Metronidazole 7.5 mg/kg/dose IV q8H	Co-amoxiclav 25 mg/kg/dose (Amoxicillin component) PO q12H **Penicillin Allergy:** Clindamycin 10 mg/kg/dose IV q8H + Co-trimoxazole 4 mg/kg/dose (Trimethoprim component) IV q12H	• Consider rabies prophylaxis according to local epidemiology.
Nail Infection			

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Infections and Likely Organisms	**Suggested Treatment**		**Notes**
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Onychomycosis Dermatophytes (*Trichophyton rubrum* and *T. mentagrophytes*) Candida, moulds (rare)	Itraconazole 5 mg/kg/dose q24H for 1 week each month for 2-3 months **OR** Fluconazole 3-6 mg/kg/week for 3-6 months		
Fungal Skin Infection			
Tinea corporis/cruris *Trichophyton rubrum* *T. mentagrophytes* *Epidermophyton floccosum*	Itraconazole 5 mg/kg/dose q24H for 2-4 weeks **OR** ***Age > 2 years:*** Griseofulvin 10-20 mg/kg/day for 2-4 weeks		
Tinea capitis Requires systemic treatment	***Age ≥2 years:*** Griseofulvin 20-25 mg/kg/day (micronized) for 6-8 weeks **OR** Itraconazole 3-5 mg/kg/dose q24H for 2-6 weeks		

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	OR Itraconazole 5 mg/kg/ dose q24H for 1 week each month for 2-3 months		
Tinea pedis/manuum	Itraconazole 5 mg/kg/ dose q24H for 2-4 weeks		